

Announcement

Kindly note: We're currently experiencing a high volume of inquiries and calls, potentially leading to delays in our initial response.



Dermatology 23S Module 2

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Day 5 Scenario 3

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[◀ Day 4 follow on Questions](#)[Day 3 scenario 2 ▶](#) Display replies in nested form**Day 5 Scenario 3**

by [Derek Kayanja \(Tutor\)](#) - Tuesday, 21 November 2023, 2:42 PM

Hi group,

Here is the last scenario of the week.

A 62-year-old man attends clinic frustrated with the lack of progress with regard to his irritating rash. He has attended the clinic several times, having seen different people, and all have suggested topical steroid therapy with topical emollients, yet he has noticed minimal improvement in this rash. This rash has now been present for 12 months and he feels it is deteriorating. He has recently had to give up working as a mechanic as his skin disease is having a significant impact on his working life.

He has a history of type 2 diabetes for which he takes metformin and canagliflozin. He also takes amlodipine and simvastatin.

On examination, this rash is present:



Please answer all of the questions below:

1. What is the likely diagnosis?
2. What would be the most appropriate management plan for the patient?
3. What histological feature is likely to be present?

154 words

[Reply](#)



Re: Day 5 Scenario 3

by [Sophie Holloran](#) - Wednesday, 22 November 2023, 7:54 AM

1. What is the diagnosis?

This is a widespread, erythematous rash with papules and plaques. It looks like there are some areas of excoriation. The rash is not symmetrical and does not cross the midline. I would also want to see the limbs, palms and soles, nails, scalp and mucosal surfaces.

In a patient with this presentation, I would initially be thinking about inflammatory dermatoses such as eczema or psoriasis. From the case, it would be worth checking that emollients and topical steroids were being used correctly. I would want a potent topical steroid such as Mometasone 0.1% for the trunk.

The red flags from this case would be the duration of the rash without any improvement (if we assume correct topical treatment use) and that the rash is having a significant impact on his life.

My suspicion would be that this is a cutaneous lymphoma such as Mycosis Fungoides.

2. What would be the most appropriate management plan?

The most useful investigation at this stage would be a skin biopsy.

I recently went to a lecture about this topic, and I was advised that the best practice would be to send multiple punch biopsies from lesions of different maturity. It is likely that the tissue will need various immunohistochemical stains and so one single punch biopsy will probably not be enough tissue to run all of the necessary tests.

I would probably arrange a basic panel of blood tests - FBC, U+E, LFT - this may become useful for any treatments that need to be started.

If the biopsy confirmed a lymphoma then this patient would go on to have staging imaging and possibly lymph node biopsies.

3. What histological feature is likely to be present?

There are a very large number of cutaneous lymphomas, each with different IHC staining and histological appearances.

For mycosis fungoides, the key feature is infiltration of malignant CD4+ T cells.

Question for the group: What is the treatment for MF?

References:

1. Oakley (2018) Mycosis fungoides pathology - Derm Net NZ. Available at: <https://dermnetnz.org/topics/mycosis-fungoides-pathology> (Accessed 22/11/23)
2. Ngan (2006) Mycosis fungoides - Derm Net NZ. Available at: <https://dermnetnz.org/topics/mycosis-fungoides> (Accessed 22/11/23)

357 words

Reply



Re: Day 5 Scenario 3

by [Xiao Liey Ding](#) - Wednesday, 22 November 2023, 11:47 AM

Hi Sophie,

I concur with you that the most likely diagnosis is Mycosis Fungoides.

Mycosis fungoides is manageable but not curable. There are no universally accepted treatment protocols, and the choice of treatment should be influenced by the patient's preferences, disease stage, available options, and local medical expertise. Recurrence is common following most interventions, and a watch-and-wait strategy may be considered for early and limited disease (Ngan, V., 2006).

Various treatment modalities exist, including:

- Topical therapies such as steroids, chemotherapy (e.g., nitrogen mustard, carmustine), and bexarotene.
- Procedural treatments, including phototherapy (PUVA, narrowband UVB), radiotherapy, and whole-body total skin electron beam therapy, as well as extracorporeal photopheresis.
- Systemic treatments encompass chemotherapy (with mycosis fungoides displaying relative chemoresistance), immunotherapy (such as interferon-alpha), and oral retinoids and rexinoids (e.g., bexarotene).
- Additional therapeutic options involve denileukin diftitox, monoclonal antibodies, and histone deacetylase inhibitors.

Reference:

Ngan, V. (2006) Mycosis fungoides - DermNet NZ. [online]. Available at: <https://dermnetnz.org/topics/mycosis-fungoides> (Accessed 22/11/23)

152 words

Reply



Re: Day 5 Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:37 PM

Xiao Liey, thank you for your post and treatment modalities.

10 words

Reply

**Re: Day 5 Scenario 3**by [Siddharth Saluja](#) - Wednesday, 22 November 2023, 12:45 PM

Hi Sophie,

Further to his post, if we do consider skin biopsy with multiple punch biopsies, would you consider any particular depth of the skin lesion or consider a normal punch biopsy. In other words, how would you ensure that you have covered a sufficient enough depth to cover a specific lesion for the biopsy to be accurate diagnostic reflection of the lesion?

Thanks.

65 words

[Reply](#)**Re: Day 5 Scenario 3**by [Sophie Holloran](#) - Wednesday, 22 November 2023, 7:31 PM

Hi Siddharth,

For a punch biopsy it depends on where you take it from - for example, the skin on the back is really thick so you'd have no problem going all the way down to the hilt on the punch. However, on places like the lower legs or head/neck, you'd want to be more careful. You can always go further in, but you can't undo what you've already done! From my understanding as long as you are getting a good sample of the epidermis and dermis I think that is fine.

92 words

[Reply](#)**Re: Day 5 Scenario 3**by [Siddharth Saluja](#) - Thursday, 23 November 2023, 11:36 AM

Thanks Sophie, explains my doubts.

5 words

[Reply](#)**Re: Day 5 Scenario 3**by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:34 PM

Thank you Sophie, very good post!, you worked through the differentials spotted the read flags and arrived at the correct diagnosis.

22 words

[Reply](#)**Re: Day 5 Scenario 3**by [Felica Mwinji Namukonde](#) - Sunday, 26 November 2023, 5:52 PM

Treatment of Mycosis Fungoides:

primary goal of treatment is palliation of symptoms which include;

pruritus, burning or cosmetic concerns and also minimize extent of disease in skin compartment to reduce risk of disease progression

Supportive measures is use of emollients, topical corticosteroids and oral antihistamines

Stage IA MF includes patches, plaques or papules involves, 10% of skin lymph nodes are not involved or viscera use of skin directed therapy rather than systemic therapy is recommended which include

Topical corticosteroids, high or very high potency are used, response is at least seen in 3 months if no response is seen end of 3 months change therapy.

Topical Mechlorethamine, alkylating agent used in first-line and second line therapy used for MF 4 to 6 months or until complete clearance.

Topical carmustine

Topical bexarotene

Topical imiquinoid

Phototherapy for MF characterized by patches only with PVUA

Localized radiation

Stage IB/IIA disease includes, patches plaques or papules that involves > or equal to 10% not involving lymph nodes or viscera

Stage IIA includes patients with patches, plaques or papules of any size with reactive palpable lymph nodes (N1) or isolated scattered neoplastic cells.

Skin directed therapies include

Topical corticosteroids, mechlorethamine, total skin electron beam therapy localized radiation and phototherapy.

systemic therapies include oral bexarotene, low-dose methotrexate, interferons, oral vorinostat, brentuximab vedotin though these are not frequently used in early stage MF in first line therapy but can be used if

there is extensive skin involvement like;

predominant plaque disease

blood involvement

not well controlled disease

patients with no access to skin directed therapies like TSEBT

patients with worse prognosis profile.

Response assessment is based on general clinical criteria like visible lesion disappearance, partial regression and relief of symptoms.

References

Hoppe R T, Kim Y H, Horwitz S (2023) *Uptodate* Available at:

https://www.uptodate.com/contents/treatment-of-early-stage-ia-to-ii-a-mycosis-fungoides?search=Treatment%20of%20early%20stage%20mycosis%20fungoides&source=search_result&selectedTitle
Accessed (26/11/23)

298 words

Reply



Re: Day 5 Scenario 3

by [Rahima Abdulaziz Ahmed](#) - Thursday, 23 November 2023, 1:40 PM

Q1;

Mycosis fungoides ; given the history of the patient; (Ngan and Writer 2006)

-Age ;62yrs, male

-white patient (lighter skin tone fitzpatrick 1,2)

-long standing irritating rash

On Examination; the poorly defined , irregular , fine scaling erythematous fine patches of variable sizes, on the sun protected sites , the breasts , trunk .

Q2:

Appropriate management plan for the patient ;

Mycosis fungoides is treatable but not curable. From the evaluation of the patient image shared, patient looks to be in the patch stage of the disease (early stage), hence management can include; (Vaidya and Badri 2023) ►

1. Topical steroids - more potent formulations , personally would compound Clobetasol cream with an antiitch cream e.g. aveeno anti itch relief lotion to apply bid x 2/52 and then reduce the frequency depending on the patients response.

2. Systemic antihistamines medications e.g. Fexofenadine 120mg nocte x 1/12

3. Topical bexarotene, imiquimod,

4. Psoralen-ultraviolet A (PUVA) therapy or ultraviolet B (UVB) therapy.

5. Systemic therapies such as retinoids or low dose methotrexate are used if skin directed therapies fail or if skin symptoms are extensive/ severe or if patients has a worse prognostic profile such as early blood involvement, folliculotropic mycosis fungoides or large cell transformation.

6. Not forgetting liaising with the endocrinologist, oncologist and counsellor for the holistic management of the patient including optimizing diabetic, hypertensive medications and addressing the emotional and psychosocial impact of the skin disease and its effect on the patients occupation.

7. Educating the patient about the nature of mycosis fungoides , the treatment plan and the importance of adherence to both dermatological and systemic therapies.

8. Addressing work-related issues; e.g. by collaborating with the patient's employer to explore work modifications or accommodations that might allow him to continue working despite the skin condition.
9. Regular Follow-ups; to monitor disease progression, treatment response and overall health.

Q3;

Histological feature likely to be present ;(Vaidya and Badri 2023)

Histopathology of mycosis fungoides varies within the stages of the disease. Superficial lymphoid infiltrate, epidermotropism with absent or rare spongiosis, and lymphoid atypia are the predominant features. For this patient, the following will be present;

1. A lymphocytic infiltrate may be seen around the basal cell layer. The cells are usually small, well-differentiated lymphocytes with rounded or cerebriform nuclei.
2. The epidermis shows hyperkeratosis and acanthosis.
3. Basal cell degeneration with melanin pigment incontinence may be seen.
4. A dense, mixed perivascular infiltrate may be seen, along with fibrosis of dermal papillae.

My question to the group; what's the staging of mycosis fungoides?

References;

1. Ngan, V. and Writer, S. (2006) Mycosis fungoides *DermNet* Available at :<https://dermnetnz.org/topics/mycosis-fungoides> (Accessed on 23/11/23)
2. Vaidya, T and Badri, T (2023) Mycosis Fungoides *StatPearls* Available at :<https://www.ncbi.nlm.nih.gov/books/NBK519572/> (Accessed on 23/11/23)

447 words

Reply



Re: Day 5 Scenario 3

by [Jillian Simpson](#) - Thursday, 23 November 2023, 5:07 PM

Diagnosis

Given the presence of well demarcated annular patches you may consider discoid eczema, tinea corporis or cutaneous T cell lymphoma as differentials.

There are several types of T cell lymphoma with mycosis fungoides (MF) and Sezary syndrome (SS) being the most common variants (Chowdhury, Griffiths & Finlay, 2022). The peak incidence of MF occurs in the 5th and 6th decades of life and is more common in men than in women, which fits with this patient (Chowdhury, Griffiths & Finlay, 2022).

Typically it occurs in non-sun exposure areas (such as the buttocks and breasts) and presents as one or more erythematous patches of variable size and shape with a fine scale (Burge, Wallis, Martin, 2016). Given all this information and the fact that it has not improved with topical steroids, mycosis fungoides is the most likely diagnosis.

Management

Clinical suspicion should be confirmed by biopsy. You may need multiple elliptical skin biopsies for histology, immunophenotyping, and molecular studies, looking for T-cell receptor gene rearrangement (Burge, Wallis, Martin, 2016). Multiple biopsies may be needed over time to confirm diagnosis especially early on in the disease (Ngan & Mitchell, 2006).

Histology

In MF, the histopathology is characterised by infiltrates of malignant T cells, with irregular cerebriform nuclei. The clinical stages: patch, plaque and tumour, correlate with the progressive density of the malignant T cells. (Chowdhury, Griffiths & Finlay, 2022).

References:

- Chowdhury, M.M.U., Griffiths, T.W. and Finlay, A.Y. (2022). *Dermatology training : the essentials*. Hoboken, NJ: Wiley-Blackwell, John Wiley & Sons, Inc. p176-177
- Burge, S.M., Wallis, D. and Martin, R. (2016). *Oxford handbook of medical dermatology*. Oxford: Oxford University Press. p358.
- Ngan, V., Mitchell, G. (2006). Mycosis fungoides | *DermNet NZ*. [online] Available at:

<https://dermnetnz.org/topics/mycosis-fungoides>. [Accessed 23/11/23].

285 words

Reply



Re: Day 5 Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:51 PM

Thank you Jillian, good post often multiple elliptical skin biopsies and the opinion of an experienced dermatopathologist is required to confirm the diagnosis.

23 words

Reply



Re: Day 5 Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:38 PM

Thank you Rahima, good post.

5 words

Reply



Re: Day 5 Scenario 3

by [Zarmeena Khan](#) - Saturday, 25 November 2023, 10:45 AM

Hi Rahima I agree with the diagnosis of mycosis fungoides, I have not seen a case in my practice but reading on group's responses have been very helpful, As for your question, Staging in mycosis fungoides is based on the extent and severity of the disease, and the most commonly used staging system is the TNMB system. Here's an overview of the TNMB staging for mycosis fungoides:

1. T Stage (Skin involvement):

- T1: Limited patches or plaques covering less than 10% of the skin surface.
- T2: Patches, plaques, or both covering 10% or more of the skin surface.
- T3: Tumors (nodules or tumors) of any size.

2. N Stage (Node involvement):

- N0: No lymph node involvement.
- N1: Involvement of one or more lymph nodes.

3. M Stage (Visceral involvement or metastasis):

- M0: No evidence of visceral involvement or metastasis.
- M1: Evidence of visceral involvement or metastasis.

4. B (Blood) Category:

- This category is used to indicate the presence or absence of abnormal lymphocytes in the blood.
- B0: No abnormal lymphocytes in the blood.
- B1: Abnormal lymphocytes present in the blood.

The staging system is combined to give an overall stage, such as stage IA, IB, IIA, IIB, III, or IV, depending on the extent of skin involvement, lymph node involvement, and the presence of visceral involvement or metastasis.

Mycosis fungoides has four stages:

1. Premycotic phase: You may develop a scaly skin rash on parts of your body that are not usually exposed to the sun, such as your lower belly, thighs, buttocks, and chest.
2. Patch phase: The skin around the rash becomes thin, and it may be itchy and dry like eczema.
3. Plaque phase: Small, raised or hard bumps form on the skin.

4. Tumor phase: Tumors, which are raised areas of skin that penetrate more deeply than plaques, develop on your skin. These tumors commonly appear on your thighs, groin, armpits, and the inside of your elbow. The tumors may develop ulcers and get infected.



Spectrum of cutaneous lesions observed in mycosis fungoides. **A**, Typical early patch with erythema and mild scale. **B**, Typical plaque, with raised, palpable borders, central clearing, and overlying scale. **C**, Large tumor with necrosis and ulceration. **D**, Generalized erythroderma. (UFO Themes, 2016)

It's important to note that mycosis fungoides is a chronic condition, and the disease may progress over time. Staging helps guide treatment decisions and prognosis.

References:

- 1.UFO Themes (2016). *Mycosis Fungoides*. [online] Oncohemakey. Available at: <https://oncohemakey.com/mycosis-fungoides/> [Accessed 25 Nov. 2023].
- 2.Olsen E, Vonderheid E, Pimpinelli N, et al. *Revisions to the staging and classification of mycosis fungoides and Sezary syndrome: a proposal of the International Society for Cutaneous Lymphomas (ISCL) and the cutaneous lymphoma task force of the European Organization of Research and Treatment of Cancer (EORTC)*. Blood. 2007 Sep 15;110(6):1713-22. doi: 10.1182/blood-2007-03-055749. PMID: 17540844.
- 3.Willemze R, Jaffe ES, Burg G, et al. *WHO-EORTC classification for cutaneous lymphomas*. Blood. 2005 May 15;105(10):3768-85. doi: 10.1182/blood-2004-09-3502. PMID: 15692063.

499 words

Reply



Re: Day 5 Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Saturday, 25 November 2023, 3:17 PM

Thank you Zarmeena, good post and good images

8 words

Reply



Re: Day 5 Scenario 3

by [Oyeombiliyetu Jason](#) - Sunday, 26 November 2023, 5:17 PM

I would like to answer the question posed:

Staging of MF is assessed using ISCL-EORTL criteria, aiding in understanding disease extent and guiding management. Determined through physical exam, biopsy, imaging, and blood tests. Poor prognosis indicators include T stage, extracutaneous disease, age >60, male gender, large cell transformation, folliculotropism, and tumor distribution at diagnosis.

T (Skin)		N (Nodes)	
T1	Limited patch/plaque ($< 10\%$ of total skin surface)	N0	No clinically abnormal LNs
		N1	Clinically abnormal LNs; histopathology Dutch grade 1 or NCI LN0-2 (clone +/-)
T2	Generalized patch/plaque ($\geq 10\%$ of total skin surface)	N2	Clinically abnormal LNs; histopathology Dutch grade 2 or NCI LN3 (clone +/-)
T3	Tumors		
T4	Generalized erythroderma	N3	Clinically abnormal LNs; histopathology Dutch grade 3- or NCI LN4 (clone +/-)
		Nx	Clinically abnormal LNs, no histopathology info
M (Viscera)		B (Blood)	
M0	No visceral involvement	B0*	No significant blood involvement
M1	Visceral involvement	B1†	Low blood tumor burden
		B2‡	High blood tumor burden

• CD4/CD8 ratio ≥ 10 (setting of an expanded CD4), or CD4+CD7- cells $\geq 40\%$ or CD4+CD26- $\geq 30\%$ of lymphocytes, or other aberrant expression of pan T-cell markers

(S. Horwitz 2008)

Reference

1. S. Horwitz et al (2008). 'Review of the treatment of Mycosis fungoides and Sezary Syndrome: a stage based approach'. *Journal of the National Comprehensive Cancer Network*. 6(4):436-442. Available at: https://www.researchgate.net/publication/5418508_Review_of_the_Treatment_of_Mycosis_Fungoides_ar_Based_Approach. Accessed: 26 November 2023.
2. American Joint Committee on Cancer. Primary Cutaneous Lymphomas. *AJCC Cancer Staging Manual*. 8th ed. New York, NY: Springer; 2017: 967-972. Available at: <https://amp.cancer.org/cancer/types/s>. Accessed: 26 November 2023.
3. T. Vaidya et al (2023). 'Mycosis Fungoides'. *National Library of Medicine*. Available at: <https://www.ncbi.nlm.nih.gov/books/NBK519572/>. Accessed: 26 November 2023.

140 words

Reply



Re: Day 5 Scenario 3

by [Pooran Outar](#) - Friday, 24 November 2023, 12:15 AM

Mycosis fungoides is characterized by well-defined, red, pruritic patches of variable size, often accompanied by atrophic (thin, wrinkled) skin (Ngan, 2006).

In this case, I recommend a **combined therapeutic approach** with the following options:

- Phototherapy: Specifically, NB-UVB, which has demonstrated favorable responses, particularly in primarily patch vs plaque disease and among patients with lighter skin types (Vaidya and Badri, 2021).
- Low Dose Methotrexate: This serves as a viable systemic option; however, close monitoring for hepatotoxicity is imperative.
- Oral Bexarotene: While effective, caution is advised as it may induce reversible hypertriglyceridemia and hypercholesterolemia, conditions from which the patient already suffers.

- Targeted Therapies: Romidepsin and Brentuximab vedotin can be considered as second-line treatments if the initial approach proves ineffective (Khan, Noor, and Horwitz, 2021).

From the image, it appears to be in the patch stage, so the following **histological features** are likely to be present (Vaidya and Badri, 2021):

- Lymphocytic infiltrate around basal cell layer.
- Small, well-differentiated lymphocytes with rounded or cerebriform nuclei.
- Epidermis: Hyperkeratosis and acanthosis.
- Basal cell degeneration with melanin pigment incontinence.
- Dense, mixed perivascular infiltrate.
- Fibrosis of dermal papillae.

References:

- Khan, N., Noor, S.J. and Horwitz, S. (2021). How We Treat Mycosis Fungoides and Sézary Syndrome. *Clinical advances in hematology & oncology : H&O*, [online] 19(9), pp.573–581. Available at: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC9364355/>.
- Ngan, V. (2006). Mycosis fungoides | DermNet NZ. [online] dermnetnz.org. Available at: <https://dermnetnz.org/topics/mycosis-fungoides>.
- Vaidya, T. and Badri, T. (2021). *Mycosis Fungoides*. [online] PubMed. Available at: <https://www.ncbi.nlm.nih.gov/books/NBK519572/>.

238 words

Reply



Re: Day 5 Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:38 PM

Thank you Pooran, good post.

5 words

Reply



Re: Day 5 Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:48 PM

Hi Group,

As Sophie stated this is a case of Mycosis fungoides (MF) a type of Cutaneous T-cell lymphoma. The most appropriate management plan for this patient is to arrange biopsy and phototherapy via dermatology. The histological feature likely to be present is Pautrier's micro-abscesses

Mycosis fungoides (MF) is characterised by polymorphic patches and plaques usually involving limb/girdle sites, the breast, and especially the buttock area. The lesion is characterised by subtle, fine, scaly, and often slightly atrophic (wrinkled) erythematous patches, which may be associated with pruritus but patients are often asymptomatic. The diagnosis of MF must be based on a combination of clinical and pathological features, including Pautrier's micro-abscesses amongst others. Clinico-pathological criteria for the early diagnosis of MF have been published by the International Society for Cutaneous Lymphoma. Multiple elliptical skin biopsies and the opinion of experienced dermatopathologists are often required to make a diagnosis. All patients with MF should have a full clinical examination and adequate diagnostic biopsies for histology as well as immunophenotypic and preferably molecular studies, because patients with a detectable T-cell clone (clonal t-cell receptor rearrangement) have a shorter duration of response and a higher rate of treatment failure. The current consensus for therapy of MF is that patients with early-stage disease should receive skin-directed therapy consisting of topical therapy, phototherapy, or radiotherapy. Systemic therapies are used for patients with early-stage disease that is resistant to skin-directed therapy, or for those with advanced disease as is the case here.

245 words

Reply

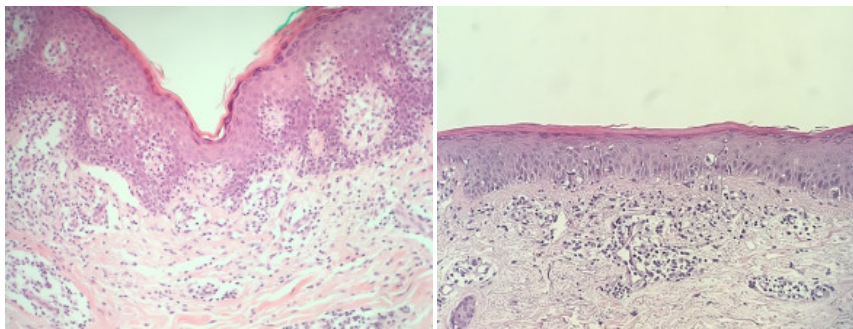

Re: Day 5 Scenario 3

 by [Arwaf Benrawwaf](#) - Friday, 24 November 2023, 6:17 PM

1. The likely diagnosis is Mycosis fungoides: a type of cutaneous T-cell lymphoma that affects the skin. It is characterized by a chronic rash with variable morphology, which may present as patches, plaques, nodules, or tumors. The rash typically develops on sun-exposed areas of the body and may be itchy or painful. As the disease progresses, it can involve lymph nodes and internal organs, leading to systemic symptoms such as fever, weight loss, and fatigue. Diagnosis is based on clinical presentation, histopathology, immunophenotyping, and molecular studies.(Willemze, R.2019)

2. The management of mycosis fungoides depends on the stage and severity of the disease. Early-stage disease may be treated with topical corticosteroids, phototherapy, or skin-directed chemotherapy. Advanced-stage disease may require systemic chemotherapy, radiation therapy, or immunotherapy. Referral to a dermatologist or oncologist is recommended for further evaluation and treatment.(Olsen, E.A.2011)

3. Histologically, mycosis fungoides is characterized by the presence of atypical lymphocytes with cerebriform nuclei in the epidermis and/or upper dermis. These cells may form Pautrier microabscesses, which are collections of atypical lymphocytes within the epidermis. In addition, there is a lymphocytic infiltrate in the dermis, which may be sparse or dense depending on the stage of the disease. Immunohistochemical staining can help to confirm the diagnosis and distinguish mycosis fungoides from other cutaneous lymphomas. (Willemze, R.2019)



(dermnetnz.org. 2018)

Question : How can patients with mycosis fungoides manage their symptoms and improve their quality of life?

Reference:

-Willemze, R., Cerroni, L., Kempf, W., Berti, E., Facchetti, F., Swerdlow, S.H. and Jaffe, E.S. (2019). The 2018 update of the WHO-EORTC classification for primary cutaneous lymphomas. *Blood*, [online] 133(16), pp.1703-1714. doi:<https://doi.org/10.1182/blood-2018-11-881268>.

-Olsen, E.A., Rook, A.H., Zic, J.A., Kim, Y., Pierluigi Porcu, Querfeld, C., Wood, G.S., Marie-France Demierre, Pittelkow, M.R., Wilson, L.D., Pinter-Brown, L., Advani, R.H., Parker, S., Kim, E.J., Junkins-Hopkins, J.M., Foss, F.M., Cacchio, P. and Duvic, M. (2011). Sézary syndrome: Immunopathogenesis, literature review of therapeutic

-dermnetnz.org. (n.d.). *Mycosis fungoides pathology* / *DermNet*. [online] Available at: <https://dermnetnz.org/topics/mycosis-fungoides-pathology#:~:text=In%20mycosis%20fungoides%2C%20the%20histopathology>.

325 words

**Re: Day 5 Scenario 3**

by [Zarmeena Khan](#) - Saturday, 25 November 2023, 12:22 PM

Hi Awraf,

Patients with mycosis fungoides (MF), a type of cutaneous T-cell lymphoma, can take several steps to manage their symptoms and improve their quality of life. It's important to note that the management plan should be individualized. In my reading there were some general strategies in which patients can be facilitated to improve their quality of life, these are a combination of clinical treatments and supportive care,

1. Topical Treatments:

- Corticosteroids: Topical corticosteroids can help reduce inflammation and itching associated with skin lesions.
- Topical Retinoids: These medications may be prescribed to manage early-stage MF lesions.

2. Phototherapy:

- PUVA (Psoralen plus Ultraviolet A) or UVB Phototherapy: Phototherapy can be effective in treating skin lesions and relieving symptoms.
- Extracorporeal Photopheresis (ECP): This therapy involves removing blood, treating it with a photosensitizing drug, exposing it to ultraviolet A light, and then returning it to the body. ECP is sometimes used in more advanced cases.

3. Systemic Therapies:

- Retinoids: Oral retinoids, such as bexarotene, may be prescribed for more widespread disease.
- Biological Therapies: Biologic drugs, such as mogamulizumab, may be considered for advanced cases.
- Chemotherapy or Immunotherapy: These may be recommended in more aggressive forms of MF.

4. Symptom Relief and Supportive Care:

- Emollients and Moisturizers: Regular use of moisturizers can help alleviate dry skin and reduce itching.
- Antihistamines: These can be used to manage itching.
- Wound Care: For ulcerated lesions, proper wound care is essential to prevent infection and improve comfort.

5. Psychosocial Support:

- Living with a chronic condition can be emotionally challenging. Seeking support from friends, family, or support groups can be beneficial.
- Psychosocial support and counselling may help cope with the emotional aspects of the disease.

6. Sun Protection:

- Protecting the skin from sun exposure is crucial, as sunlight can exacerbate symptoms. Using sunscreen, wearing protective clothing, and avoiding prolonged sun exposure.

7. Regular Follow-up:

- Regular follow-up is essential to monitor the progression of the disease and adjust the treatment plan as needed.

8. Maintain a Healthy Lifestyle:

- Adopting a healthy lifestyle, including a balanced diet and regular exercise, can contribute to overall well-being.

Reference:

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473 words

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Re: Day 5 Scenario 3

by [Derek Kyanja \(Tutor\)](#) - Saturday, 25 November 2023, 3:16 PM

Hi Zarmeena, thank you this is a good response to Arwaf's question.

12 words

[Reply](#)



Re: Day 5 Scenario 3

by [Derek Kyanja \(Tutor\)](#) - Saturday, 25 November 2023, 3:15 PM

Thank you Arwaf, good post.

5 words

[Reply](#)



Re: Day 5 Scenario 3

by [Sadaf Basharat](#) - Saturday, 25 November 2023, 11:20 PM

Hi Arwaf.

I must say that you have asked a very important question, as there is no cure of Mycosis fungoides, all the treatments are aimed towards controlling the symptoms or for delaying the recurrence of the disease. However the real challenge is in trying to improve the quality of life in these patients. Mostly the most concerning symptoms of the patients is Pruritus and its really important to prescribe enough treatment to treat pruritus, if we aim to improve the quality of life.

Several agents are used to manage pruritus/itching in MF. They include

1. Emollients
2. Topical steroids: To help reduce the inflammation and itching too.
3. Oral Anti Histamines: Oral histamines are noted to help reduce the itching, where emollients alone have not shown enough improvement.

As Zarmeena has already explained that its very important to offer psychological support to these patients, as it tends to be a chronic disease and at the same time very concerning as per aesthetic point of view. Patients usually become anti social and tend to avoid going in public. There are now many support groups available for derma patients and patients for MF should also be counselled for them. Avoidance of sunlight or direct heat can also reduce the symptoms and at the same time taking care of the clothings can also make patient's life quiet better. Cotton clothings should be preferred over the synthetic/polyester fibres, as cotton fabrics tend to be less irritant . A healthy life style including regular exercise, healthy and balanced diet , avoidance to smoking and stress management can all help in improving the quality of life and help in dealing with the symptoms.

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316 words

Reply



Re: Day 5 Scenario 3

by [Derek Kayanja \(Tutor\)](#) - Sunday, 26 November 2023, 12:27 PM

Thank you Sadaf, this is a good contribution to our discussion.

11 words

Reply

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